INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Topotecan by infusion · small-cell lung cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00311
Version 4

Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.

Treatment stages

01 **Chemotherapy course**
Up to 6 cycles

02 **Review and next plan**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

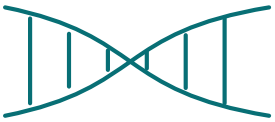
Your chemotherapy

This treatment aims to slow or shrink small-cell lung cancer that has returned after treatment, when repeating the first treatment is not appropriate. Benefit varies; your team explains the aim for you.

TOPOISOMERASE-I INHIBITOR CHEMOTHERAPY

Topotecan infusion

Hycamtin; supplied brand may vary



Disrupts deoxyribonucleic acid (the cell's genetic material) (DNA) copying and repair

Topotecan blocks topoisomerase I, an enzyme that relieves strain while DNA is copied. Unrepaired DNA breaks can stop dividing cancer cells; healthy blood-forming and gut cells can also be affected.

Medicine reaches cells → DNA damage persists → Cell division disrupted

What to watch for

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

My treatment

Evidence and limits

An older randomized trial found similar survival with intravenous topotecan and CAV chemotherapy, with improvement in several symptoms in the topotecan group. These historical groups do not predict an individual outcome or compare every treatment available today. Treatment choice also depends on previous response, time since treatment, fitness and preferences.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment course

Treatment stages

01 **Chemotherapy course**
Up to 6 cycles

02 **Review and next plan**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

UP TO 6 CYCLES

Chemotherapy course

1 2 3 4 5 6

Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.

INDIVIDUAL PLAN

Review and next plan

Review

Your team reviews benefit and side effects and agrees whether to continue, change or stop.

Relative educational plan; your team confirms actual appointments and changes.

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Check	Before treatment	During the cycle	Before next course	After treatment
Safety blood tests	Blood count, kidney and liver profile	Report symptoms; extra tests if needed	Blood count, kidney and liver review	Follow abnormal results as advised
Disease assessment	Confirm treatment aim	Symptoms reviewed	Scans or other tests on consultant plan	Agree next treatment or follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00311 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Topotecan by infusion · small-cell lung cancer: treatment and checks

Relapsed small-cell lung cancer

Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between courses	Course review
Treatment NCCP 00311	Confirm the exact treatment plan and contact numbers before starting.	Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.	Follow support plan and report symptoms	Review benefit/ toxicity; next plan
Safety tests NCCP 00311	Blood count, kidney/liver tests	Review symptoms; extra tests if needed	Blood count, kidney and liver tests before treatment and before each cycle. Scans and follow-up follow the consultant plan.	Follow abnormal results

What each check is for

1 **Before each course**

Before starting

Safety and plan may change

Ask / **act:** Team confirms next treatment

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before treatment

Confirm the exact treatment plan and contact numbers before starting.

STEP 2

Treatment visit or home course

Each infusion takes about 30 minutes. Blood review, preparation and observation add to appointment time. The consultant may prescribe only 3 or 4 treatment days in selected circumstances; follow the confirmed appointment plan.

STEP 3

Bring to review

Bring your medicine list, dose record and questions; report new symptoms before starting another course.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your 21-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Infusion visits	 **Days 1–5** intravenous (IV)
No planned chemotherapy	 **Days 6–21** Blood-count and symptom recovery varies.

Effects vary; no risk curve or guaranteed recovery day is shown.

The next cycle starts 21 days after this one, if your team confirms it.

> Do not use this page to calculate or change doses.

Between-treatment notes

Day 1–5: Treatment days

Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable. Follow the confirmed prescription; report problems promptly.

Day 6–21: Between treatments

No planned chemotherapy; continue prescribed support and monitoring. Do not restart until the team confirms the next cycle.

Support around the medicines

1 **Sickness and bowel symptoms**

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

2 **Blood tests and review**

Blood count, kidney and liver tests are checked before treatment and before each cycle. Extra tests depend on symptoms and results.

3 **Pregnancy, fertility and medicines**

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

4 **During the infusion**

Tell the nurse immediately about rash, flushing, breathing difficulty, dizziness, or pain/swelling at the drip site.

Between treatments

Keep your prescription and contact plan to hand.

Blood tests and review

Pregnancy, fertility and medicines

During the infusion

When to call

Blood counts

An infection can be serious even after treatment days have finished. A normal earlier blood test does not rule out a new problem.

Tests and review

Blood count, kidney and liver tests before treatment and before each cycle. Scans and follow-up follow the consultant plan.

Scans and tumour markers

Your consultant decides which are relevant and when. Do not assume a fixed imaging interval or that tumour markers are required for everyone.

After treatment

Agree follow-up, symptom support and the next plan before the course ends.

Questions worth asking

Why can treatment be delayed?

Blood results or symptoms may make a delay or dose change safer. Your team confirms when to restart.

Are capsules gentler than an infusion?

Route alone does not predict treatment risk. Your team explains the relevant alternatives.

When will I have scans?

Your consultant sets the timing for your disease and treatment plan. No universal scan or tumour-marker interval is specified here.

> **Try explaining the plan back:** Explain which days you take treatment, when your next blood tests are due and whom you will call tonight if unwell.

Check your exact treatment This patient guide covers the small-cell lung cancer indication and the stated route only.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1–5 infusion support

Active medicines: topotecan

Emetogenic risk: low

Medicine	Dose / route	When
Individually prescribed sickness medicine	Single agent selected under low-risk parenteral guidance	Before each treatment dose; reconcile multi-day support No duplicated preventive courses. Product interactions and patient factors apply.

Additional phase-specific support

- Follow a written prescription with start/stop days.
- granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) may be used after individual risk assessment; it is not automatically prescribed.
- Monitor infusion site and reactions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hair / energy

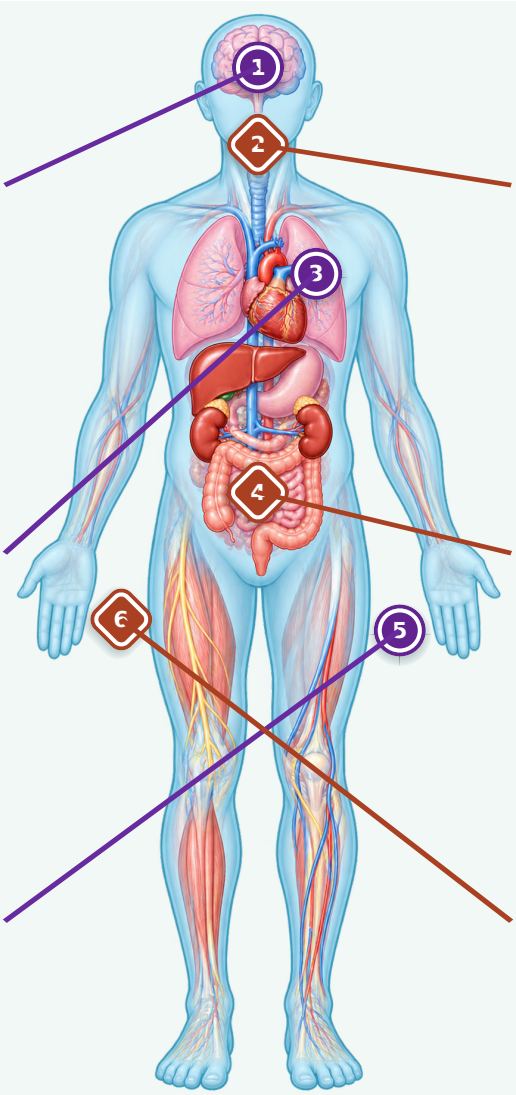
Topotecan

Hair loss and tiredness may occur; report marked weakness or dizziness.

2 Mouth

Topotecan

Tell the team about ulcers or painful swallowing; ask for help if you cannot drink.



3 Chest / breathing

Topotecan

New cough, breathlessness or chest pain needs urgent assessment.

4 Stomach / bowel

Topotecan

Report persistent diarrhoea or vomiting. Fever with severe tummy pain is urgent.

5 Skin / allergy

Topotecan

Report spreading rash; swelling or breathing difficulty needs immediate help.

6 Blood counts

Topotecan

Fever, unusual bruising or bleeding needs an immediate call. The marker is schematic.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00311 v4 · pNCCP 00311 supportive care; NCCP antiemetic guidance**
   https://healthservice.hse.ie/documents/6714/311_v4_Topotecan_Monotherapy_5day.pdf
2. **EMA Hycamtin product information**
   https://www.ema.europa.eu/en/documents/product-information/hycamtin-epar-product-information_en.pdf
3. **NCCP NCIS medical oncology antiemetic guidance V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
4. **von Pawel 1999 randomized intravenous (IV) topotecan versus CAV**
   https://pubmed.ncbi.nlm.nih.gov/10080612/